

## Breaking prolonged sitting reduces postprandial glycemia in healthy, normal-weight adults: a randomized crossover trial.

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**BACKGROUND:** Sedentary behavior is a risk factor for cardiometabolic disease. Regularly interrupting sedentary behavior with activity breaks may lower this risk. **OBJECTIVE:** We compared the effects of prolonged sitting, continuous physical activity combined with prolonged sitting, and regular activity breaks on postprandial metabolism. **DESIGN:** Seventy adults participated in a randomized crossover study. The prolonged sitting intervention involved sitting for 9 h, the physical activity intervention involved walking for 30 min and then sitting, and the regular-activity-break intervention involved walking for 1 min 40 s every 30 min. Participants consumed a meal-replacement beverage at 60, 240, and 420 min. **RESULTS:** The plasma incremental area under the curve (iAUC) for insulin differed between interventions (overall  $P < 0.001$ ). Regular activity breaks lowered values by  $866.7 \text{ IU} \cdot \text{L}(-1) \cdot 9 \text{ h}(-1)$  (95% CI: 506.0, 1227.5  $\text{IU} \cdot \text{L}(-1) \cdot 9 \text{ h}(-1)$ ;  $P < 0.001$ ) when compared with prolonged sitting and by  $542.0 \text{ IU} \cdot \text{L}(-1) \cdot 9 \text{ h}(-1)$  (95% CI: 179.9, 904.2  $\text{IU} \cdot \text{L}(-1) \cdot 9 \text{ h}(-1)$ ;  $P = 0.003$ ) when compared with physical activity. Plasma glucose iAUC also differed between interventions (overall  $P < 0.001$ ). Regular activity breaks lowered values by  $18.9 \text{ mmol} \cdot \text{L}(-1) \cdot 9 \text{ h}(-1)$  (95% CI: 10.0, 28.0  $\text{mmol} \cdot \text{L}(-1) \cdot 9 \text{ h}(-1)$ ;  $P < 0.001$ ) when compared with prolonged sitting and by  $17.4 \text{ mmol} \cdot \text{L}(-1) \cdot 9 \text{ h}(-1)$  (95% CI: 8.4, 26.3  $\text{mmol} \cdot \text{L}(-1) \cdot 9 \text{ h}(-1)$ ;  $P < 0.001$ ) when compared with physical activity. Plasma triglyceride iAUC differed between interventions (overall  $P = 0.023$ ). Physical activity lowered values by  $6.3 \text{ mmol} \cdot \text{L}(-1) \cdot 9 \text{ h}(-1)$  (95% CI: 1.8, 10.7  $\text{mmol} \cdot \text{L}(-1) \cdot 9 \text{ h}(-1)$ ;  $P = 0.006$ ) when compared with regular activity breaks. **CONCLUSION:** Regular activity breaks were more effective than continuous physical activity at decreasing postprandial glycemia and insulinemia in healthy, normal-weight adults. This trial was registered with the Australian New Zealand Clinical Trials registry as ACTRN12610000953033.